

HOW TO OPEN A HOME HEALTH AGENCY IN NEW MEXICO

A step-by-step guide to licensing, staffing, funding, and running a home health agency in New Mexico — the license that also lets you provide personal care.

What this guide is. A practical roadmap to opening a licensed home health agency in New Mexico under 8.370.22 NMAC — the rule the New Mexico Health Care Authority uses to license home health agencies. Unlike many states, New Mexico does not separately license non-medical home care; the home health agency license is the door, and a licensed agency may also provide non-medically directed personal care, homemaker, and companion services. So this is the license to get whether you plan to provide skilled care, personal care, or both.

How to use it. Read Parts 1 through 4 before you spend real money — they explain what the license authorizes and exactly how to get it, including New Mexico's distinctive letter-of-intent-and-crosswalk application and its registered-nurse supervision and 75-hour aide-training rules. Parts 5 and 6 are about revenue: where clients come from and how you get paid, including New Mexico Medicaid (Turquoise Care and Mi Via) and the state Sandata Electronic Visit Verification system. Parts 7 through 9 keep you compliant once you are open. The two worked examples show how it comes together in a metro market, and the contacts, glossary, and references at the end are your quick-reference desk. Verify every fee, figure, and phone number before you rely on it — requirements change.

The big picture

New Mexico licenses **home health agencies** under 8.370.22 NMAC through the Health Care Authority, Division of Health Improvement. There is no separate non-medical license — but a licensed home health agency **may also provide non-medically directed personal care**. You apply with a letter of intent, a functional program outline, and your policies with the rule annotated to them (a crosswalk); the fee is about \$300. You get a temporary license (up to 120 days) and then an annual license. Skilled care runs under a physician-ordered plan of care with registered-nurse supervision; aides need 75 hours of training. Most non-medical work is private pay, while skilled and waiver services are billed through New Mexico Medicaid with Sandata EVV.

The Path at a Glance

1. **Decide your model.** Skilled home health, personal care, or both — all under the one home health agency license.
2. **Form your business.** Register with the New Mexico Secretary of State, register for gross receipts tax, get an EIN, and line up insurance.
3. **Send your letter of intent.** Email a letter of intent to the Health Care Authority licensing office to start the process.
4. **Build your program and policies.** Write your functional program outline and your policies, then annotate the rule to them (the crosswalk).
5. **Apply and pay the fee.** Submit your application, functional program, policies and crosswalk, and the licensure fee (about \$300).
6. **Pass the review and survey.** The licensing authority reviews your documents and surveys your agency; you receive a temporary license (up to 120 days).
7. **Hire and screen your team.** Complete caregiver criminal history screening, employee abuse registry checks, and TB tests; train and evaluate aides.
8. **Enroll with payers.** For Medicaid, enroll with the Health Care Authority and contract with Turquoise Care plans; set up Sandata EVV.
9. **Open, then earn your annual license.** Serve clients in compliance, and convert your temporary license to the annual license.

Part 1 — What a New Mexico Home Health Agency Is

A **home health agency** in New Mexico is an agency licensed under 8.370.22 NMAC to provide home health services to clients in their place of residence. Home health includes skilled services — nursing; physical, occupational, and speech therapy; and medical social services — provided under a plan of care ordered by the client's physician and supervised by a registered nurse, along with home health aide services. Crucially, **a licensed home health agency may also provide non-medically directed services** — personal care, homemaker, and companion services that do not require a physician order.

Who oversees you

The scope line to respect

Skilled services (nursing and therapy) must run under a physician-ordered plan of care with RN supervision. Personal care, homemaker, and companion services do not need a physician order, but they are still provided by trained, competent staff under your agency's supervision and service plan. Know which service you are providing, and staff and document it accordingly.

Part 2 — Getting Licensed (8.370.22 NMAC)

Step 1 — Letter of intent

Step 2 — Functional program outline

Step 3 — Policies with a crosswalk

Step 4 — Application, fee, and survey

Step 5 — Temporary license, then annual license

Keep your crosswalk alive

The regulation-to-policy crosswalk is not a one-time chore. Keep it current as you revise policies and as the rule changes. At survey time, a current crosswalk is the fastest way to show compliance — and it keeps your whole manual honest.

Part 3 — Governance, Staffing, Screening & Training

Governance

Registered-nurse supervision

Home health aide training and competency

Screening every caregiver

Part 4 — Setting Up the Business

- **Entity & registration.** Register your LLC or corporation with the New Mexico Secretary of State (sos.nm.gov).
- **Tax.** Register for gross receipts and withholding tax with the New Mexico Taxation and Revenue Department, and get an EIN from the IRS.
- **Insurance.** Carry general and professional liability coverage appropriate to a home health agency.
- **Workers' compensation.** New Mexico requires workers' compensation coverage for employers at the statutory employee threshold (generally three or more employees — verify). Get it before you hire into that range.
- **Office.** Maintain a physical business office in New Mexico; clients are admitted only through your licensed location.

A low barrier, done right

Compared with states that impose a certificate of need or heavy net-worth tests, New Mexico's ~\$300 fee and document-driven licensure make the barrier to entry relatively low. That is an opportunity — but the agencies that thrive are the ones that treat the policy manual, screening, and supervision as real operations, not paperwork.

Part 5 — Finding Clients

- **Hospital discharge planners and case managers** arrange in-home support for patients going home — the single most valuable relationship for a new agency.
- **Skilled nursing and rehab facilities** discharge patients who still need help at home.
- **The aging network** — New Mexico's Aging and Long-Term Services Department and the Aging and Disability Resource Center (1-800-432-2080) — connects families to services.
- **Hospices, assisted living communities, elder-law attorneys, and geriatric care managers** all meet families at decision points and refer trusted providers.
- **Medicaid care coordinators** at the Turquoise Care managed care organizations authorize and coordinate services; clean documentation and EVV compliance keep referrals coming.

Part 6 — Getting Paid

Funding source	What it covers	How to access it
Private pay	Any service the client agrees to; the most common source for non-medical personal care	Your disclosed rates and a service agreement; bill the client or representative directly
Medicaid — Turquoise Care	Covered home health and personal care for eligible members, through managed care organizations	Enroll with HCA; contract with each Turquoise Care MCO; bill per its authorization; use Sandata EVV
Medicaid — Mi Via waiver	Self-directed services for eligible people with developmental disabilities or medically fragile conditions	Operate under Mi Via provider agreements; follow Mi Via documentation rules
Medicare	Skilled, intermittent home health for eligible beneficiaries (if you become certified)	Obtain Medicare certification and meet the Conditions of Participation (42 CFR Part 484)
Veterans programs	In-home support for eligible veterans	Coordinate with the VA (1-877-222-8387) and applicable community-care networks
Long-term care insurance	In-home care covered by the client's policy	Verify the client's benefits; bill per the policy's requirements

Electronic Visit Verification (Sandata)

Part 7 — Staying Compliant

- **Supervise on schedule.** Keep RN supervisory visits to clients' homes at least every 60 days (and on any Medicaid schedule that applies), documented every time.
- **Report incidents.** Document all incidents within five days (8.370.9 NMAC) and report abuse, neglect, exploitation, or misappropriation as required.
- **Report abuse to APS.** Report suspected abuse, neglect, or exploitation of an incapacitated adult to New Mexico Adult Protective Services at 1-866-654-3219 (call 911 in immediate danger).
- **Keep screening and training current.** Track caregiver screening, TB tests, aide in-service (12 hours/year), and annual performance reviews so none lapses.
- **Follow labor law.** Pay at least the applicable New Mexico or higher local minimum wage, pay overtime over 40 hours a week, and track compensable travel between clients.
- **Stay survey-ready.** Keep policies implemented, records complete, and your crosswalk current — surveys and complaint investigations are unannounced.

Part 8 — Your First 90 Days

1. **Weeks 1–2.** Finalize your onboarding: confirm screening, TB tests, and aide competency for every hire; set your supervisory-visit calendar.
2. **Weeks 3–4.** Complete Medicaid enrollment and MCO contracting if you bill Medicaid; finish Sandata EVV setup and test it.
3. **Weeks 5–8.** Introduce yourself to discharge planners, the aging network, and two or three referral partners a week; leave a simple one-page overview.
4. **Weeks 9–12.** Run your first quality review: check documentation, supervisory-visit timeliness, and any complaints; fix what you find.

Part 9 — Common Mistakes to Avoid

- **Assuming there's a non-medical license.** There isn't — the home health agency license is the path, and it also covers personal care.
- **Skipping the crosswalk.** Policies without the regulation-to-policy crosswalk stall your application; build it as you write.
- **Letting an aide start before competency.** The 75-hour training, competency evaluation, and screening must be complete first.
- **Missing 60-day supervisory visits.** Missed RN supervisory visits are a common, avoidable deficiency — schedule them.
- **Billing without EVV alignment.** For Medicaid, claims that don't match Sandata EVV records get denied or recouped.
- **Citing the old rule.** 7.28.2 NMAC was repealed; the current rule is 8.370.22 NMAC. Cite the current one.

Worked Example 1 — Albuquerque

Your first referral calls

For...	Who to contact
Presbyterian Hospital — case management / discharge	1100 Central Ave SE, Albuquerque 87106 · 505-841-1234
UNM Hospital — case management / discharge	2211 Lomas Blvd NE, Albuquerque 87106 · 505-272-2111
Lovelace Medical Center — case management / discharge	601 Dr Martin Luther King Jr Ave NE, Albuquerque 87102 · 505-727-8000
The Rehabilitation Center of Albuquerque — discharge planning	5900 Forest Hills Dr NE, Albuquerque 87109 · 505-822-6000
Canyon Transitional Rehabilitation Center — discharge planning	10101 Lagrima De Oro Rd NE, Albuquerque 87111 · 505-298-1231
Aging & Disability Resource Center (statewide front door)	1-800-432-2080 · nmaging.state.nm.us

Worked Example 2 — Las Cruces

Your first referral calls

For...	Who to contact
MountainView Regional Medical Center — case management	4311 E Lohman Ave, Las Cruces 88011 • 575-556-7600
Memorial Medical Center — case management / discharge	2450 S Telshor Blvd, Las Cruces 88011 • 575-522-8641
Three Crosses Regional Hospital — case management	2560 Samaritan Dr, Las Cruces 88001 • 575-521-3000
Aging & Disability Resource Center (statewide front door)	1-800-432-2080 • nmaging.state.nm.us
Non-Metro Area Agency on Aging (serves Doña Ana County)	Reach through the ADRC at 1-800-432-2080

Who to Call — New Mexico Quick Directory

For...	Who to contact
HCA — Health Facility Licensing & Certification (licensing)	facility.license@hca.nm.gov · Program Operations Bureau, Santa Fe 505-476-9093
HCA — Medical Assistance Division (Medicaid provider enrollment)	hca.nm.gov · New Mexico Medicaid (Turquoise Care)
Sandata — New Mexico EVV support	Through the HCA EVV program · hca.nm.gov
NM Department of Workforce Solutions (labor standards / minimum wage)	1-877-664-6984 · dws.state.nm.us
NM Workers' Compensation Administration	1-800-255-7965 · workerscomp.nm.gov
NM Secretary of State (business registration)	1-800-477-3632 · sos.nm.gov
NM Taxation and Revenue Department (gross receipts tax)	1-866-285-2996 · tax.nm.gov
Adult Protective Services (report abuse/neglect/exploitation)	1-866-654-3219 (24/7) · 911 if in immediate danger
NM Children, Youth & Families Dept (child abuse reporting)	1-855-333-7233
Aging & Disability Resource Center (aging network)	1-800-432-2080 · nmaging.state.nm.us
Eldercare Locator (national)	1-800-677-1116 · eldercare.acl.gov

Glossary

- **8.370.22 NMAC** — New Mexico's current rule setting the requirements for home health agencies (effective July 1, 2024; replaced 7.28.2 NMAC).
- **Health Care Authority (HCA)** — the New Mexico agency that licenses home health agencies (through the Division of Health Improvement) and administers Medicaid (through the Medical Assistance Division).
- **Functional program outline** — the description of your scope of services, expected client volume, and contracted services submitted at licensure.
- **Crosswalk** — a copy of the licensing rule annotated to your policies, showing which policy satisfies each regulation.
- **Temporary license** — the initial license issued on approval, valid up to 120 days (no more than two consecutive), before the annual license.
- **Plan of care** — the physician-ordered written plan directing a client's skilled services, reviewed and signed by the physician.
- **CCHSP / EAR** — the Caregivers Criminal History Screening (8.370.5 NMAC) and the Employee Abuse Registry (8.370.8 NMAC), both checked before direct care.
- **Turquoise Care** — New Mexico's Medicaid managed-care program (renamed from Centennial Care on July 1, 2024).
- **Mi Via** — New Mexico's self-directed Medicaid waiver for certain individuals with developmental disabilities or medically fragile conditions.
- **Sandata** — the state-provided Electronic Visit Verification system, free to New Mexico provider agencies.

Core Rules & References

- 8.370.22 NMAC — Requirements for Home Health Agencies (effective July 1, 2024).
- 8.370.3 NMAC — Health facility licensure fees and procedures.
- 8.370.2 NMAC — Adjudicatory hearings; 8.370.4 NMAC — sanctions and civil monetary penalties.
- 8.370.5 NMAC — Caregivers Criminal History Screening; 8.370.8 NMAC — Employee Abuse Registry.
- 8.370.9 NMAC — Incident reporting, intake, processing, and training.
- 8.325.9 NMAC — Medicaid home health services; 42 CFR Part 484 — Medicare Conditions of Participation.
- Public Health Act, 24-1-1 et seq. NMSA 1978; Health Care Authority Act, 9-8-6 NMSA 1978.
- New Mexico Minimum Wage Act and federal FLSA; NMSA 1978 Chapter 52 (workers' compensation).

This guide is general educational information, not legal, tax, or accreditation advice. New Mexico rules, fees, programs, and phone numbers change — verify each with the New Mexico Health Care Authority and the relevant payer or authority before you rely on it.